

DISCHARGE SUMMARY - Discharge Summary - 5

A female with the past medical history of Ewing sarcoma, iron deficiency anemia, hypertension, and obesity.

DATE OF ADMISSION:

MM/DD/YYYY.

DATE OF DISCHARGE:

MM/DD/YYYY.

REFERRING PHYSICIAN:

AB CD, M.D.

ATTENDING PHYSICIAN AT DISCHARGE:

X Y, M.D.

ADMITTING DIAGNOSES:

1. Ewing sarcoma.
2. Anemia.
3. Hypertension.
4. Hyperkalemia.

PROCEDURES DURING HOSPITALIZATION:

Cycle seven Ifosfamide, mesna, and VP-16 chemotherapy.

HISTORY OF PRESENT ILLNESS:

Ms. XXX is a pleasant 37-year-old African-American female with the past medical history of Ewing sarcoma, iron deficiency anemia, hypertension, and obesity. She presented initially with a left frontal orbital swelling to Dr. XYZ on MM/DD/YYYY. A biopsy revealed small round cells and repeat biopsy on MM/DD/YYYY also showed round cells consistent with Ewing sarcoma, genetic analysis indicated a T1122 translocation. MRI on MM/DD/YYYY showed a 4 cm soft tissue mass without bony destruction. CT showed similar result. The patient received her first cycle of chemotherapy on MM/DD/YYYY. On MM/DD/YYYY, she was admitted to the ED with nausea and vomiting and was admitted to the Hematology and Oncology A Service following her first course of chemotherapy. She had her last course of chemotherapy on MM/DD/YYYY followed by radiation treatment to the ethmoid sinuses on MM/DD/YYYY.

HOSPITAL COURSE:

1. Ewing sarcoma, she presented for cycle seven of VP-16, ifosfamide, and mesna infusions, which she tolerated well throughout the admission.
2. She was followed for hemorrhagic cystitis with urine dipsticks and only showed trace amounts of blood in the urine throughout the admission.